

Acuity is one part of vision

Measure each eye separately and distinguish refractive blur from other visual dysfunction.

A useful first assessment

Finding	Meaning / action
Distance blur improves with pinhole	An uncorrected refractive component is likely.
Near blur, distance clear	Test near acuity with the appropriate reading correction.
Cannot read largest letter	Progress through counting fingers, hand movements and light perception.
6/6 with a field complaint	Normal central acuity does not establish normal peripheral vision.

Record what was tested

Document the eye, distance, aided or unaided status and pinhole result. Use distance spectacles for a distance chart. The denominator identifies the smallest line read, while additional letters are recorded separately. A chart result is a measure of clarity at the tested distance; colour, fields and binocular function still need assessment when the history points to them.

Afferent and efferent pupils

Equal resting pupils can coexist with a relative afferent pupillary defect.

Read the response pattern

What happens when the light is moved?

Both pupils relatively dilate with one eye illuminated

That eye supplies weaker afferent input: an RAPD.

One pupil fails with either eye illuminated

Consider that pupil's efferent pathway or iris response.

Both pupils constrict equally with either eye illuminated

No relative afferent asymmetry is demonstrated by this test.

Technique and interpretation

Use dim surroundings, a distant fixation target and comparable illumination of each eye for about three seconds. Observe both pupils when the light is swung. If the suspected pupil is fixed, a reverse RAPD may be sought in the reactive fellow pupil. Compare colour or red saturation monocularly when visual quality differs, rather than relying on acuity alone.

Fields and the fundus

Keep fixation steady during field testing and use the vessels to find the optic disc.

Choose the examination for the complaint

Test	Technique	What it adds
Confrontation	One eye; same eye height; target midway	Central and peripheral field screening
Amsler grid	Near correction; one eye; central-dot fixation	Central distortion or missing areas
Fundoscopy	Corresponding eye and hand; distant fixation	Red reflex, optic disc, vessels and retina
Fluorescein	Cobalt-blue illumination	Corneal epithelial staining

Avoid a false normal result

A patient who follows the peripheral target is no longer testing the intended field. An Amsler grid is abnormal if lines are distorted even when its corners remain visible. During ophthalmoscopy, trace retinal vessels toward their convergence to locate the disc; inspect the macula briefly with the patient looking toward the light.

Make the history explain the result

Clarify tempo and monocular versus binocular symptoms before naming a cause.

Four discriminating questions

Onset

Was loss gradual, or sudden with delayed presentation?

Task affected

Is the difficulty at distance, near, centrally or in the peripheral field?

Associated symptoms

Ask about pain, redness, double vision, previous eye disease or surgery.

Double vision

Does duplication persist with one eye alone, or disappear when either eye is covered?

Combine history and examination

Monocular diplopia directs attention to optical or ocular causes; binocular diplopia suggests misalignment. An unexplained discrepancy in acuity or field behaviour needs a complete examination and careful repeat testing. Fogging or other consistency tests can demonstrate preserved visual capacity, but they do not establish a patient's intent. Organic disease must be excluded before concluding functional visual loss.